A VISUAL INVESTIGATION · MALAYSIA · 2015–2024

Malaysia's *Hospital Divide*

12 VISUAL CLUES ACROSS 6 ACTS

Why public beds tell only half the story.

BEGIN THE INVESTIGATION

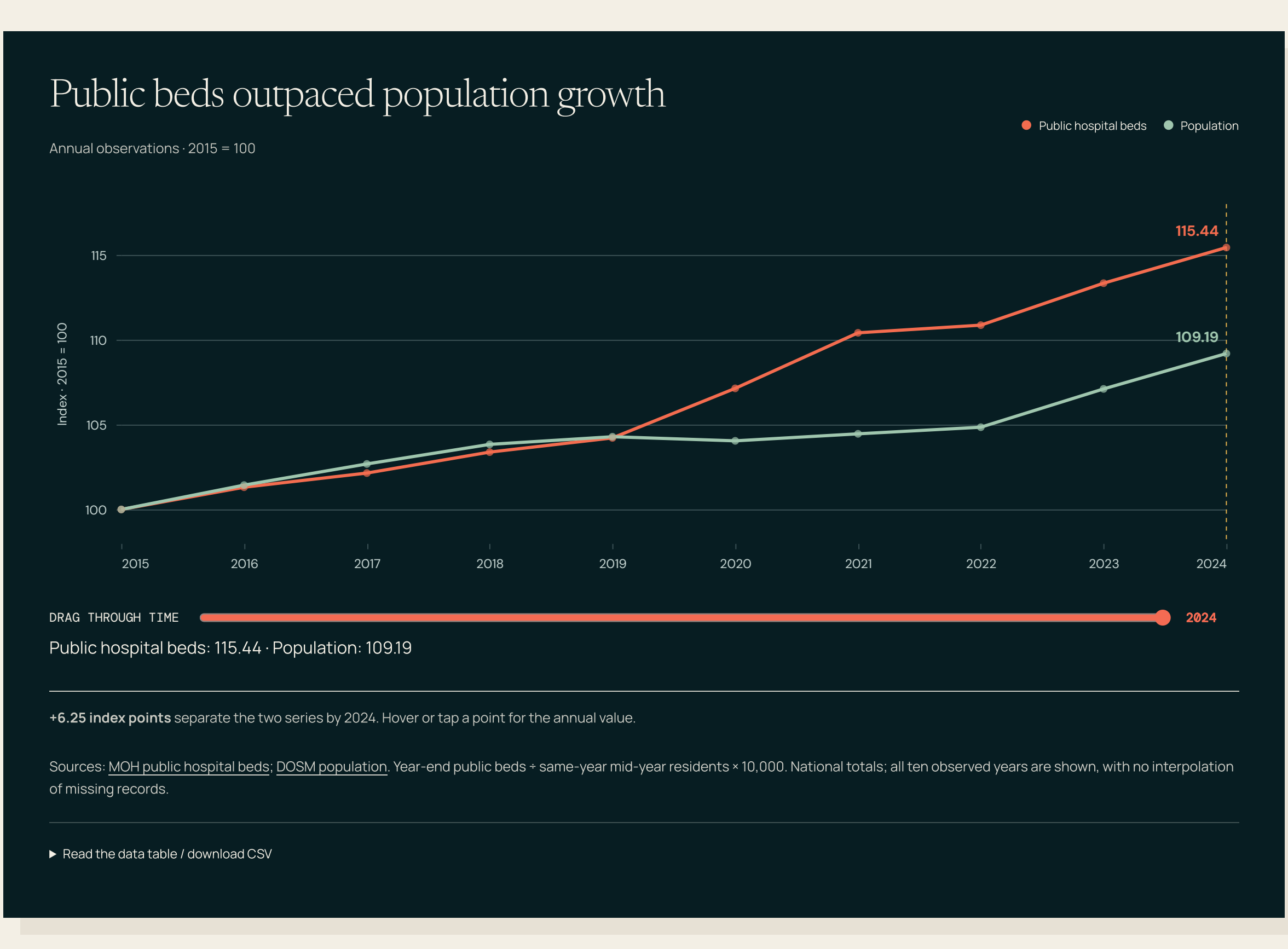
ACT I · THE NATIONAL DIVIDE

From a distance, the system appears to grow.

Public hospital beds grew 15.44% between 2015 and 2024, while population grew 9.19%. The national gain is real—but it does not describe every state.

Public beds outpaced population growth

Annual observations · 2015 = 100



DRAG THROUGH TIME 2024

Public hospital beds: 115.44 · Population: 109.19

+6.25 index points separate the two series by 2024. Hover or tap a point for the annual value.

Sources: MOH public hospital beds; DOSM population. Year-end public beds ÷ same-year mid-year residents × 10,000. National totals; all ten observed years are shown, with no interpolation of missing records.

▶ Read the data table / download CSV

READ CAREFULLY
Observed annual data. Population is context, not a direct measure of hospital demand.

ACT II · GEOGRAPHY CHANGES EVERYTHING

A national average has no address.

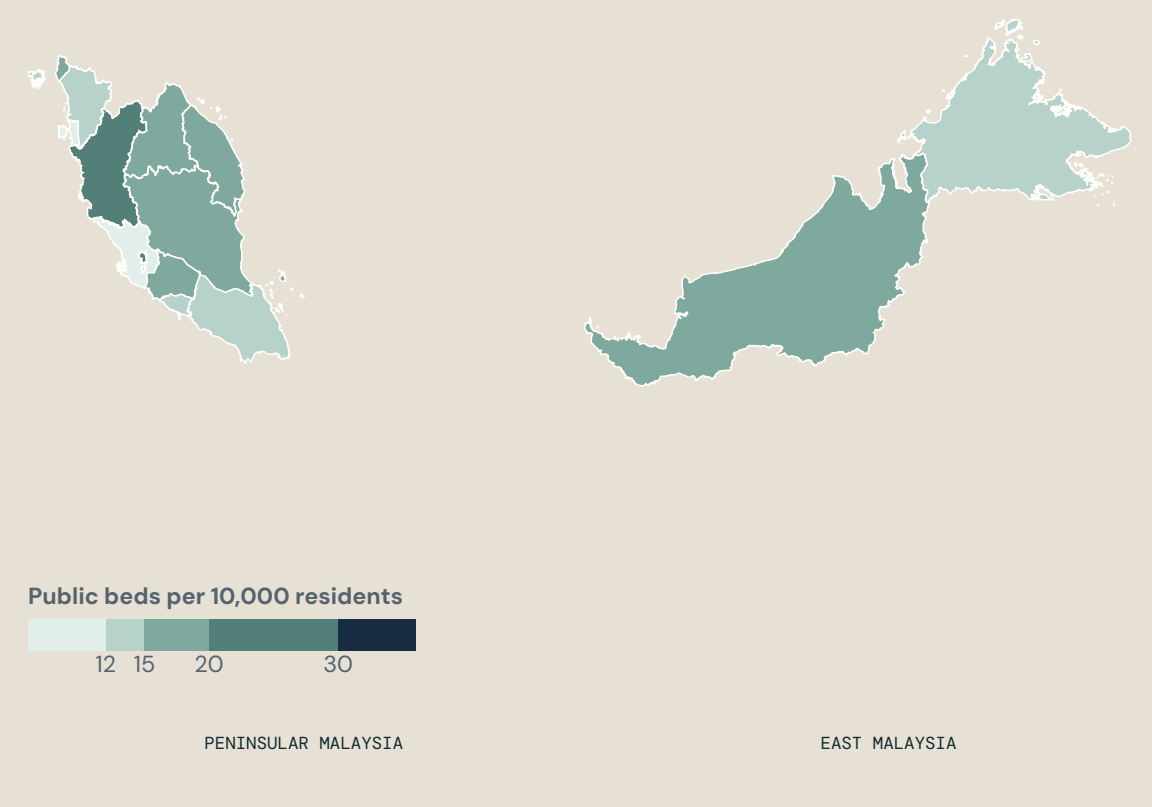
Capacity is experienced locally. Select a state to carry it across the map, timeline, and later comparisons.

Public beds per 10,000 residents

All states and federal territories · 2024

Highlight a state / territory: All 16 states & territories
Map view: All Malaysia

Hover or tap a state for its rate. Use the state selector to read small federal territories; the regional views enlarge the same boundaries.



Selangor **11.32** Perak **23.43** Putrajaya **70.16**

Resident population is not a hospital catchment. Putrajaya's small denominator produces a high rate; these data do not measure patient referral flows.

Sources: MOH public hospital beds; DOSM population. Year-end public beds ÷ same-year mid-year residents × 10,000. Boundaries: DOSM administrative GeoJSON, effective vintage undocumented. Sarawak topology repaired in a separate copy; original coordinates retained except the documented intersection repair. This is a statistical display, not a legal boundary reference.

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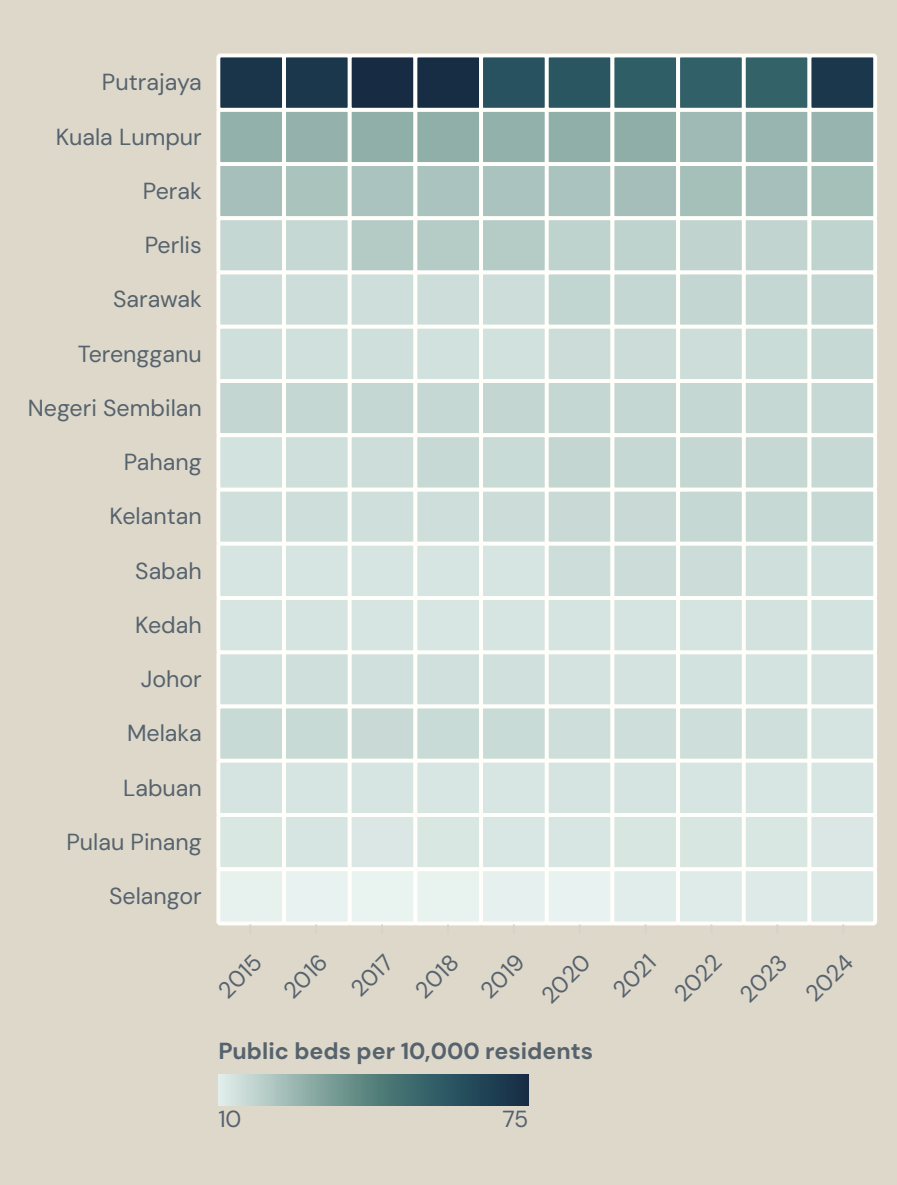
How did state differences evolve?

Public beds per 10,000 residents · 2015–2024

Highlight a state / territory: All 16 states & territories



7 of 16 states and territories had lower public-bed rates in 2024 than in 2015. Rows are ordered by their 2024 rate; a single linear colour scale preserves comparisons across the decade.

Sources: MOH public hospital beds; DOSM population. Year-end public beds ÷ same-year mid-year residents × 10,000. Each cell is an observed state-year pair. Hover or read the table for precise rates, including differences compressed by Putrajaya's high rate.

▶ Read the data table / download CSV

ACT III · THE HIDDEN PRIVATE SECTOR

Now reveal the beds the first map left out.

Private capacity does not simply add more of the same. It clusters, stretches the distance between states, and can reorder the national picture.

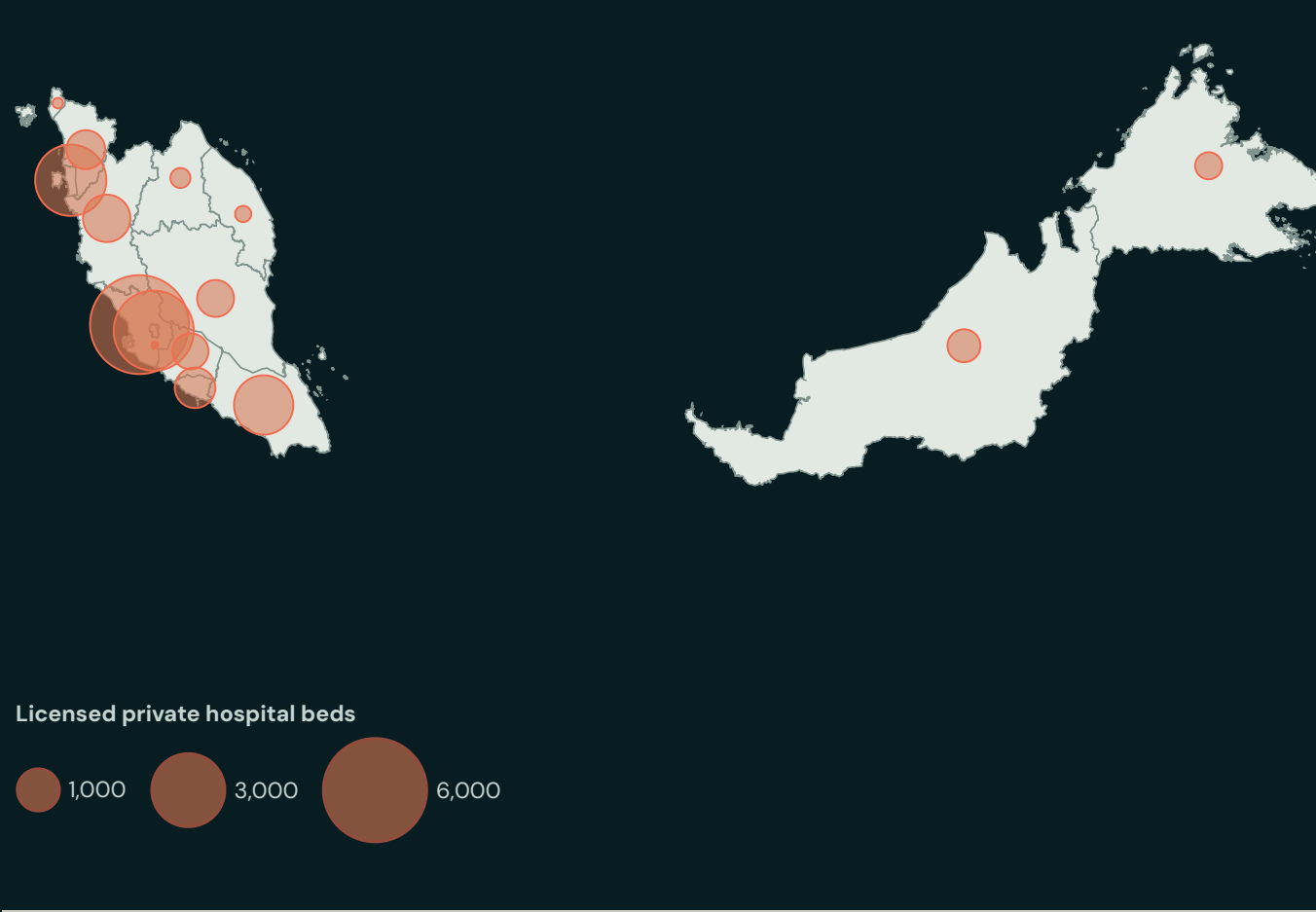
CHART 04 THE PRIVATE SECTOR

Private beds are concentrated.

19,920 licensed private hospital beds change the picture. Nearly 60% sit in Selangor, Kuala Lumpur and Pulau Pinang.

These circles represent state totals, not individual hospital locations or affordable access.

Map view: All Malaysia



Pulau Pinang: 2,847 · Selangor: 5,496 · Kuala Lumpur: 3,602
Together, these three account for **59.96%** of Malaysia's 19,920 licensed private hospital beds. Circle **area** represents bed count; symbols overlap in the Klang Valley. Use the regional view and table to inspect small territories. Labuan reports zero beds, so has no visible circle.

Source: Health Indicators 2025, table 4.4 (2024). Licensed private hospitals only, excluding maternity/nursing homes and hospices. DOSM boundaries: existing validated geometry, vintage/reuse caveats unchanged. Symbols sit inside each state's largest polygon; they are state totals, not hospital locations. Counts are not population-adjusted or operational capacity.

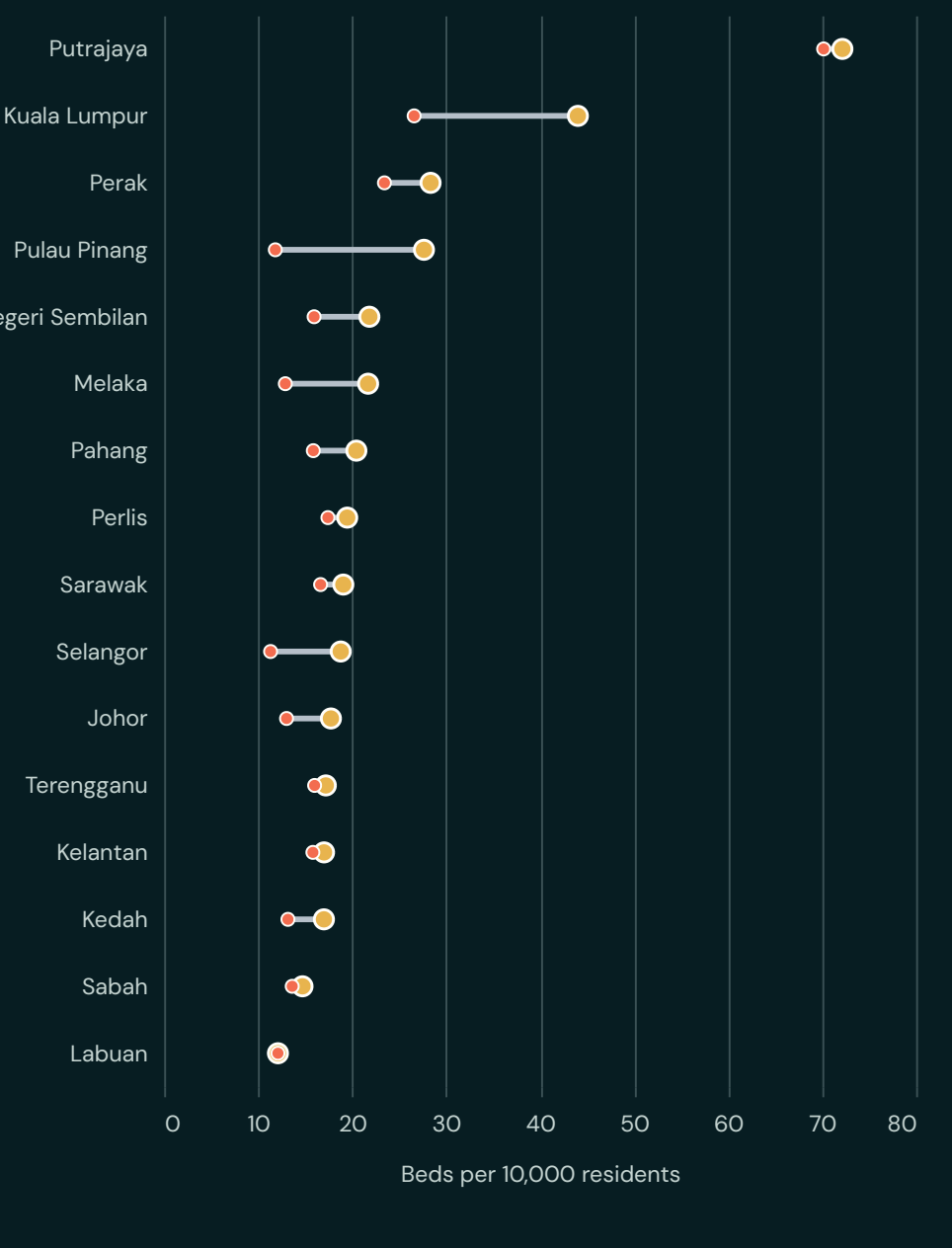
▶ Read the data table / download CSV

Public versus combined hospital beds

Beds per 10,000 residents · all 16 states and federal territories · 2024

Highlight a state / territory: All 16 states & territories
COMBINED
Order states by: Combined capacity

The line shows the addition of licensed private beds. "Private contribution" orders by private share of combined capacity. Highlighting keeps all 16 rows visible.



Pulau Pinang: 11.82 → 27.64. Selangor: 11.32 → 18.78. The same resident denominator is used for both endpoints; Labuan's endpoints coincide because it reports zero private hospital beds.

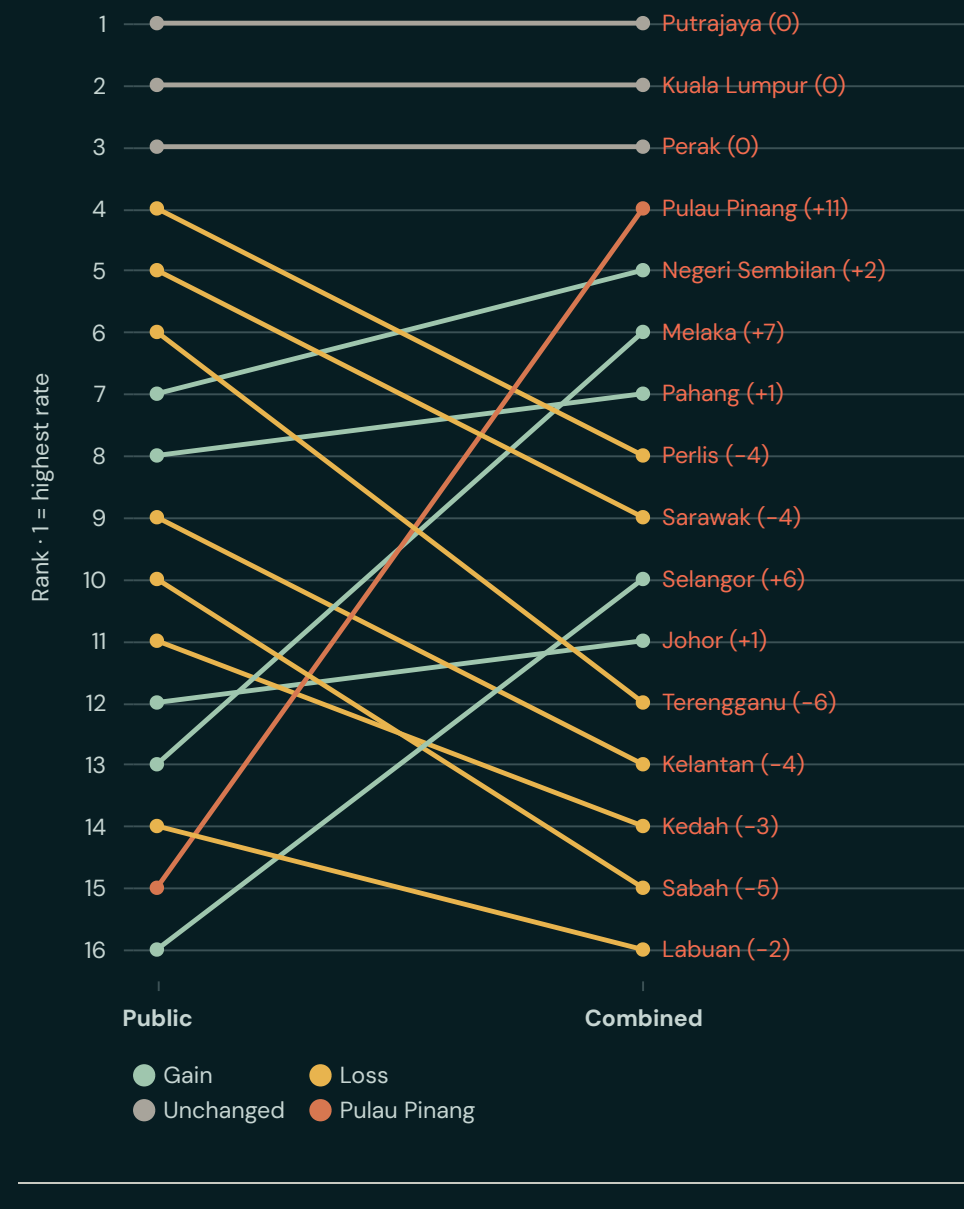
Sources: MOH public hospital beds; DOSM population. Year-end public beds ÷ same-year mid-year residents × 10,000. Private beds: Health Indicators 2025, table 4.4, printed p136 (2024 observations). Combined = public hospital/special-institution beds + licensed private hospital beds; maternity homes, nursing homes, hospices and ambulatory centres excluded. Rank 1 is the highest resident-adjusted rate.

▶ Read the data table / download CSV

Private beds reorder the rankings

All 16 states and territories · 2024 · rank 1 = highest beds per 10,000

RESET RANKS
Highlight a state / territory: All 16 states & territories



Pulau Pinang gains 11 places: 15th → 4th. Upward lines show rank gains; downward lines show losses. Labels show places gained (+) or lost (−). These are two definitions of capacity in the same year, not change over time. A rank loss does not mean beds were removed.

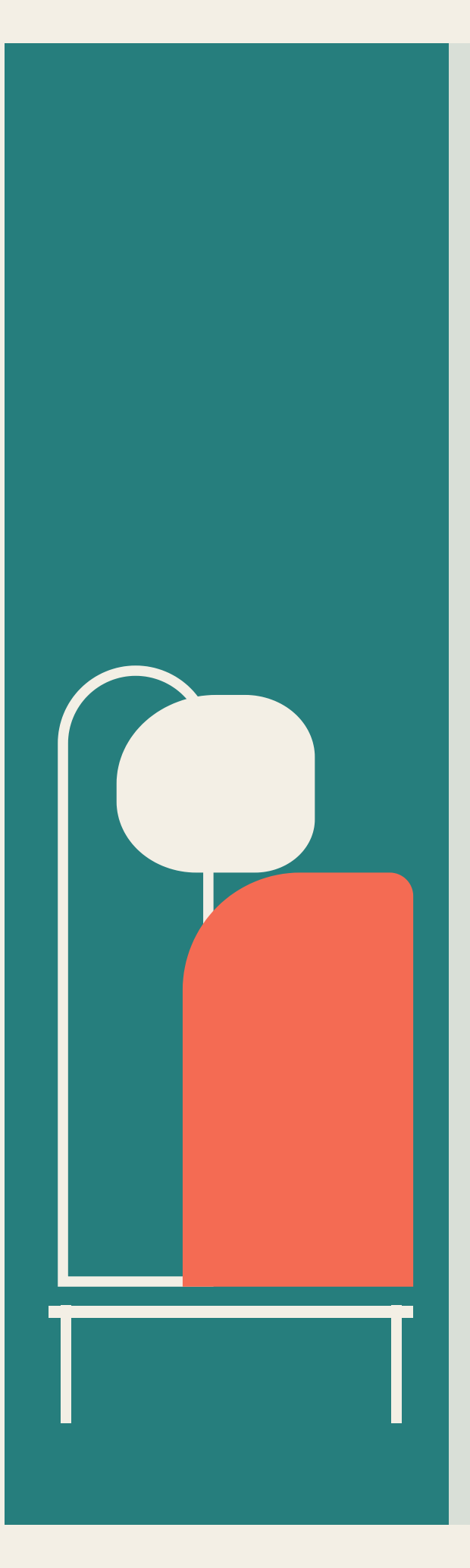
Sources: MOH public hospital beds; DOSM population. Year-end public beds ÷ same-year mid-year residents × 10,000. Private capacity: Health Indicators 2025, table 4.4. Rankings reuse the validated public and combined rates; licensed private hospitals only. This chart compares rank, not the size of rate differences.

▶ Read the data table / download CSV

ACT IV · CAPACITY VERSUS AVAILABILITY

A bed can exist on paper and still be out of reach.

Occupancy introduces operational pressure. Physical stock, staffing, referrals, turnover, and demand all sit between a count and an available bed.

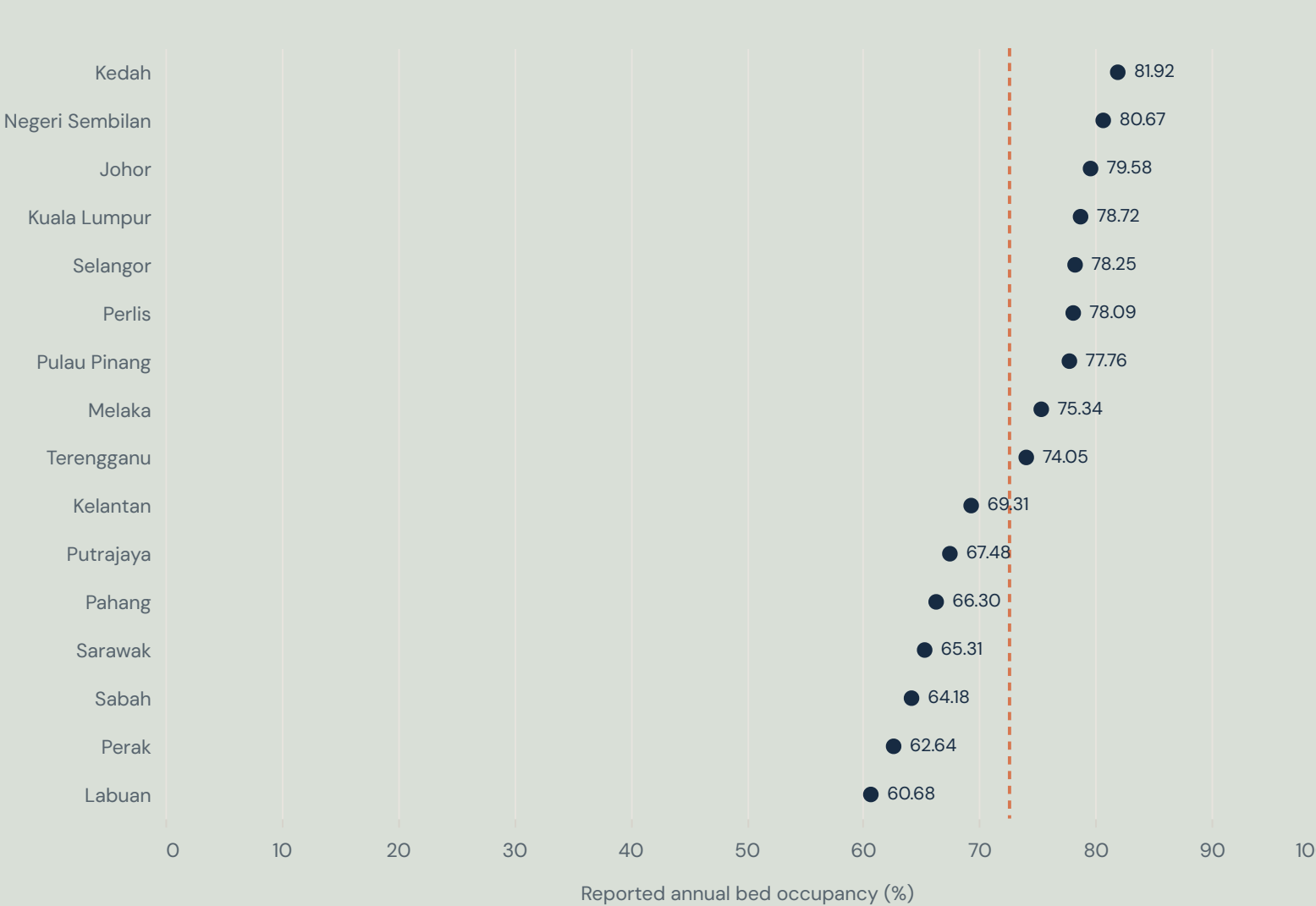


Annual MOH bed occupancy

Reported annual rate · all 16 states and federal territories · 2024

Highlight a state / territory: All 16 states & territories

● National MOH annual occupancy: 72.61%



Source: Health Indicators 2025, table 4.6, printed pp139–151. MOH hospitals and special institutions only; excludes other-government and private hospitals. The national reference is the published national rate, not an unweighted mean of states. This is historical annual utilisation, not current daily ICU pressure.

▶ Read the data table / download CSV

ACT V · FOLLOWING THE PATIENTS

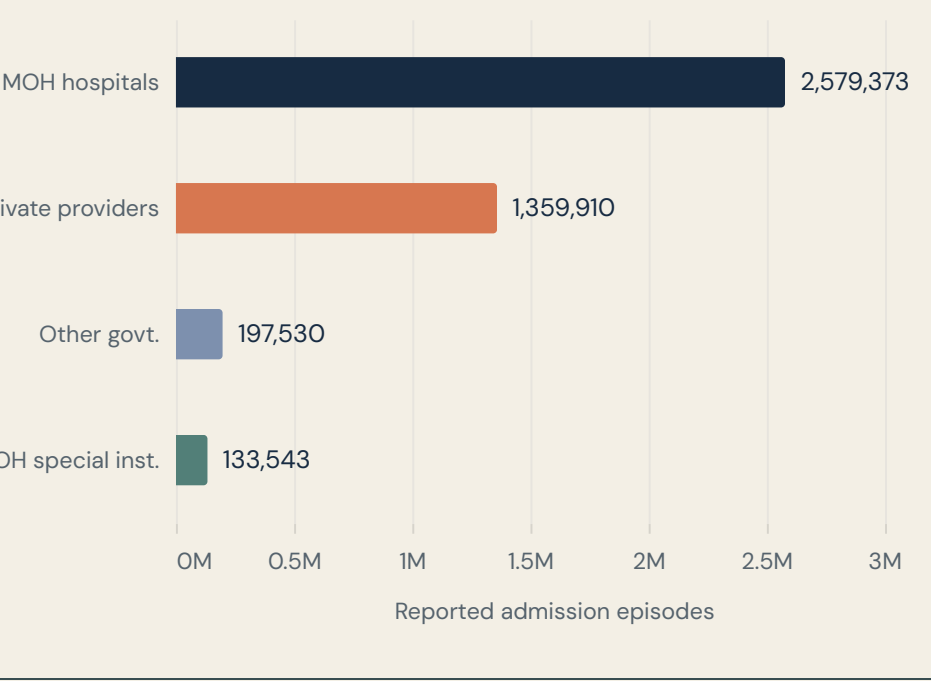
Where does hospital activity actually happen?

Counts rank provider groups; rectangle areas show their shares of the same reported total. Admissions are episodes, not unique patients. Reporting coverage differs across providers.

Reported admissions by provider

Malaysia · admission episodes · 2024

ADMISSIONS SHARE (%)



Reporting coverage: MOH 97.94% submission; private sector 82.33% response. No adjustment has been made for missing reports.

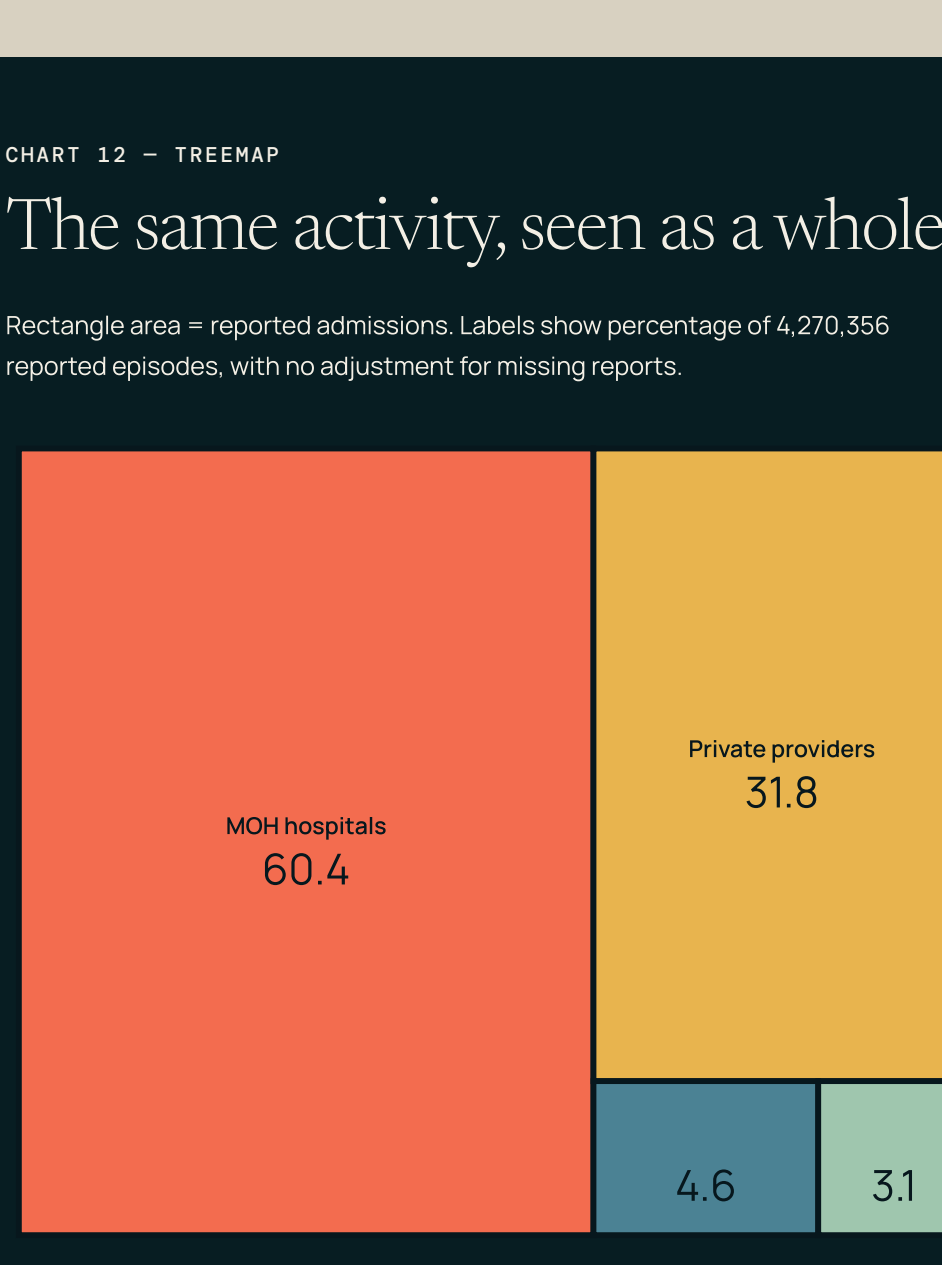
Source: Health Indicators 2025, table 4.13, printed p159. Private admissions include hospitals, maternity homes, nursing homes and hospices. These counts are broader than the hospital-only private bed figures above. Admissions are episodes, not unique people; totals do not establish resident demand.

▶ Read the data table / download CSV

CHART 12 — TREEMAP

The same activity, seen as a whole.

Rectangle area = reported admissions. Labels show percentage of 4,270,356 reported episodes, with no adjustment for missing reports.



🟥 MOH hospitals	2,579,373 · 60.40%
🟨 Private-sector providers	1,359,910 · 31.85%
🟦 Other government hospitals	197,530 · 4.63%
🟩 MOH special institutions	133,543 · 3.13%

Source: Health Indicators 2025, table 4.13 (2024). Same four mutually exclusive provider groups as the bar chart. MOH submission 97.94%; private response 82.33%. Private providers include hospitals, maternity/nursing homes and hospices. Areas describe reported composition, not complete demand or unique patients.

Download chart data (CSV)

ACT VI · THE PRESSURE AHEAD

The demand story is changing shape.

An ageing share is a headline. The full population pyramid compares two observed age structures—and shows why tomorrow's needs cannot be inferred from today's bed count.

POPULATION AGED 65+

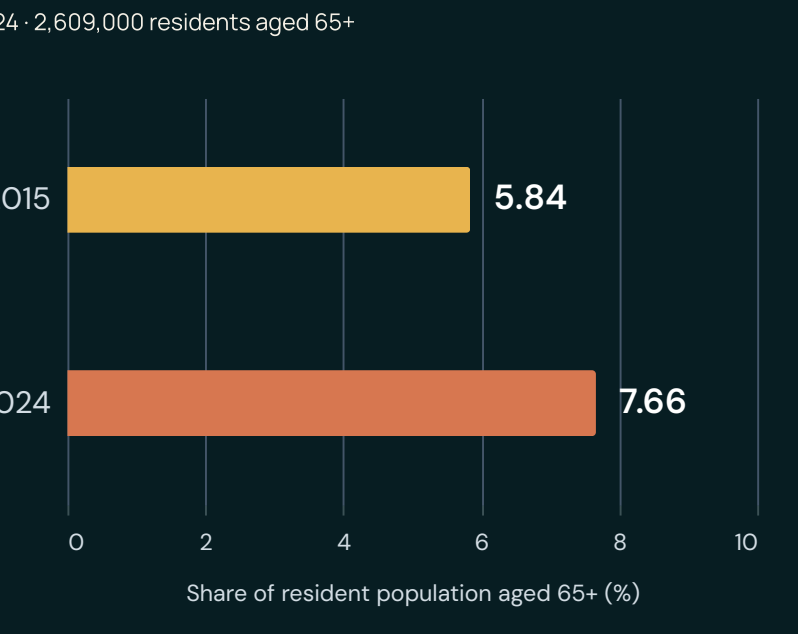
Malaysia is getting older. The increase is gradual, but its direction is clear.

5.84%

2015 · 1,821,500 residents aged 65+

7.66%

2024 · 2,609,000 residents aged 65+



Source: DOSM national population estimates. Both sexes, overall ethnicity; ages 65–69 through 85+ divided by the same year's all-age total. Age structure is context, not an estimate of hospital demand.

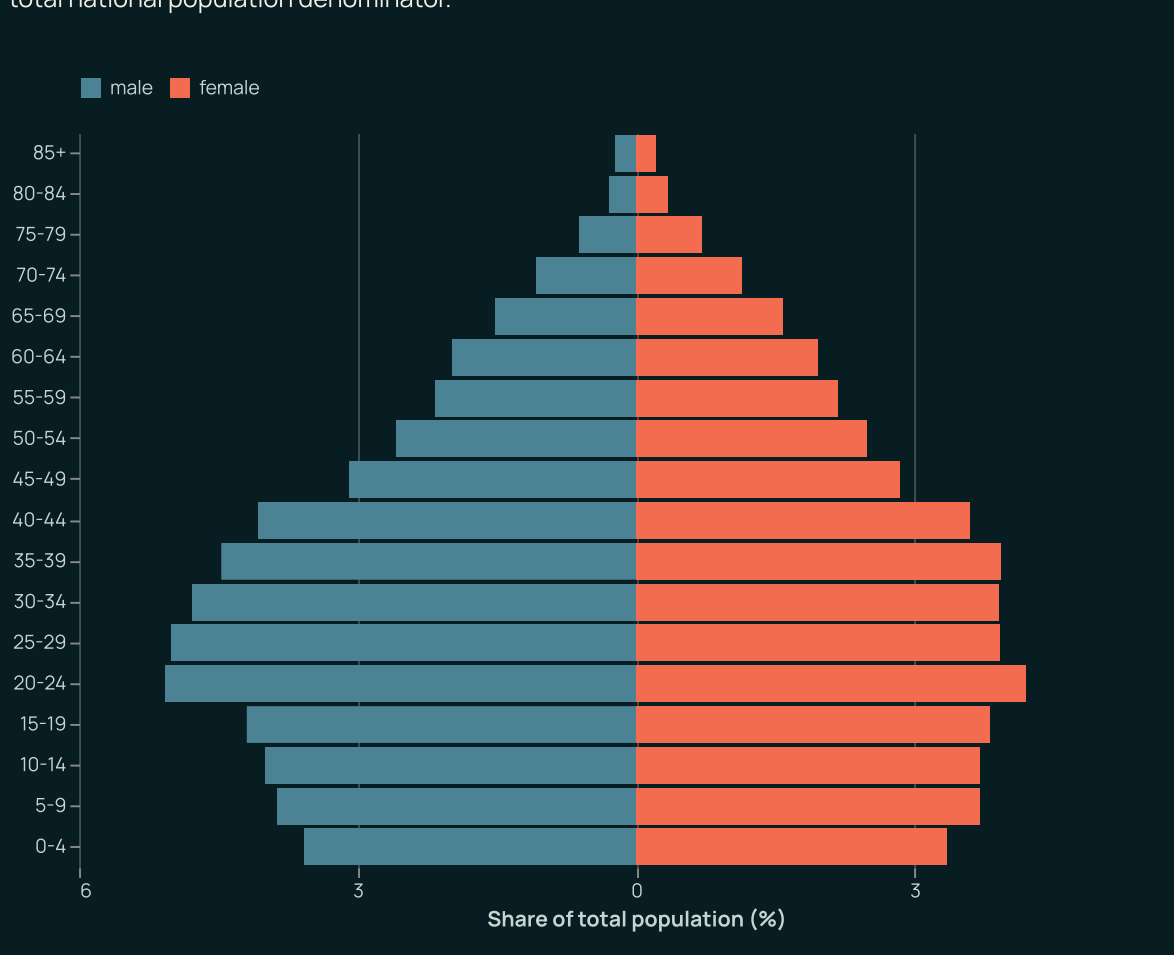
▶ Read the data table / download CSV

CHART 11 — POPULATION PYRAMID

A population changing shape.

2015 2024

Male residents extend left; female residents extend right. Both years use the same scale and their own total national population denominator.



Source: DOSM national population, overall ethnicity, 18 matching age bands; 85+ is open-ended. Published rounded bins retained; sums differ from sex totals by −100 to +300 residents. These are observed cross-sections, not tracked cohorts or forecasts.

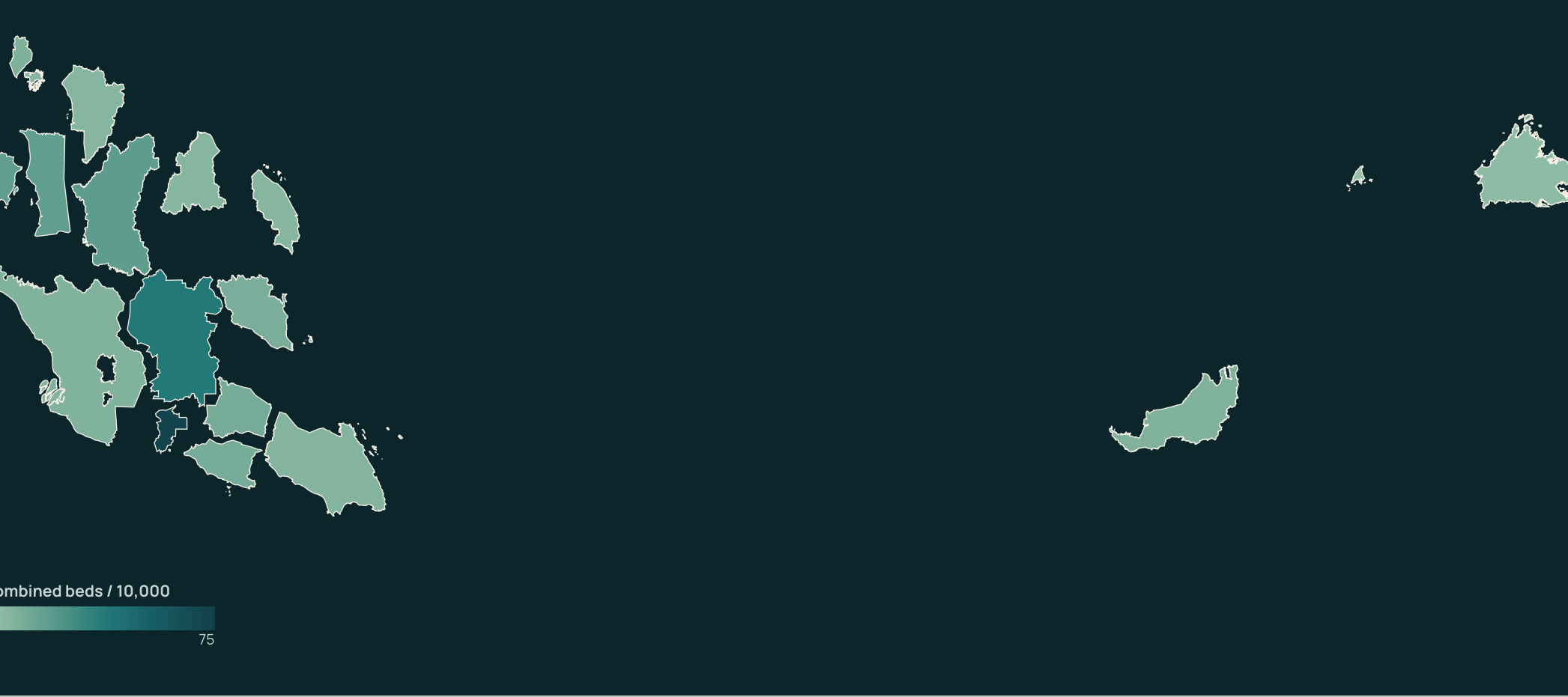
Download chart data (CSV)

CHART 10 — NON-CONTIGUOUS CARTOGRAM

What if land yielded to capacity?

GEOGRAPHY CAPACITY
Highlight a state: All 16 states & territories

Shape area = combined hospital beds. Shapes are uniformly resized and separated from their geographic positions. Colour = combined beds per 10,000 residents.



All 16 states and territories contribute to 71,968 combined beds. Small territories can be selected for precise tooltips. Displaced shapes do not measure distance or access.

Sources: MOH public beds, Health Indicators 2025, table 4.4, DOSM boundaries. Equal-area cylindrical projection; uniform scaling followed by deterministic collision separation. Original datasets unchanged; Sarawak repair and undocumented boundary vintage caveats remain. Public official and private licensed beds have different operational definitions.

Download chart data (CSV)

THE FINDING IS NOT A NUMBER

More beds do not necessarily mean ***equal access, available capacity,*** or sufficient care for the future.

Bed counts are a starting point—not a verdict on access or quality. Geography, provider mix, occupancy, activity, and demography each redraw the picture.

SOURCES & METHODOLOGY

Observed MOH and DOSM data through 2024. Public beds include special institutions; private beds cover licensed hospitals. Occupancy is annual and MOH-only. Admissions have incomplete reporting. Ageing is context, not a demand forecast.

RETURN TO THE BEGINNING

MALAYSIA'S HOSPITAL DIVIDE · TAN HONG GUAN · 35100621 · FIT3179

Sources:
MOH PUBLIC BEDS
·
HEALTH INDICATORS 2025
·
DOSM POPULATION
·
DOSM BOUNDARIES

Rates = beds ÷ resident population × 10,000. Combined beds = public hospital/special-institution beds + licensed private hospital beds. Private admissions include maternity homes, nursing homes and hospices, unlike hospital-only capacity. Geographic vintage is undocumented; Sarawak is repaired in a separate derived copy. Full source notes and data downloads accompany the charts.

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